



It sits on the clinician's calendar marked pending, it counts as committed load, and the remaining balance is on screen. Nothing lives on a sticky note.

Every physician order, recertification and resumption of care re-enters this loop — and each one is a new auth question



- Every payer, plan and state sets its own — there is no single rule to learn
- Indiana Medicaid — 8 visits shared across PT / OT / ST
- Indiana Medicaid — 30 days from the discharge date, not the admit date
- Ohio Medicaid — caps similarly; commercial and MA plans differ again

- Auth is permission — how many visits the payer will allow
- PDGM is payment — the economic band for the period
- LUPA is the floor; utilisation management is the ceiling
- A visit can be authorised and still be uneconomic

The data already exists. The auth team writes the payer's rules into a coordination note at verification — days before anyone writes the plan of care.

Surfacing those rules at plan-of-care creation is the highest-value, lowest-complexity win in this flow — and a patient-care win, because abrupt discharges happen when nobody planned for the real visit budget.



Traditional Medicare has no pending auth

Auth governs permission · PDGM governs payment

~~—Pending-auth visits are invisible~~
~~—not on a calendar, not counted~~

~~-50 pending-auth workflows a day~~
~~so schedulers bulk-clear without reading~~

~~Plans of care ignore payer limits~~
~~then the team is surprised~~

~~Abrupt discharge when the visit budget quietly runs out~~

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Every failure this sheet used to end on is answered above: auth is derived rather than keyed, and the pending visit stays visible and counted.

The payer's budget is shown at plan-of-care creation, and the balance is on screen long before it runs out.